Competitive Landscape: plaque psoriasis (EU, 2022)

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Market Overview

In the EU plaque psoriasis market, 4 brands compete for first-line+ patients as of Q3 2022. The segment is shaped by guideline updates, reimbursement decisions and real-world evidence.

The number of branded options in plaque psoriasis has grown since 2020, increasing pressure on standard of care positioning.

Positioning: Humivex

In the EU plaque psoriasis market, Humivex maintains a 33% market share as of Q2 2022, with the strongest uptake among dermatologists.

Sentiment index for Humivex in EU: 0.74 (scale 0 to 1, 2022). Clinicians view the brand strongly positively; long-term safety data are still limited.

Positioning: Adalibra

In the EU plaque psoriasis market, Adalibra maintains a 16% market share as of Q2 2022, with the strongest uptake among dermatologists.

Sentiment index for Adalibra in EU: 0.55 (scale 0 to 1, 2022). Clinicians view the brand moderately positively; infusion logistics add to treatment burden.

Positioning: Cosentex

In the EU plaque psoriasis market, Cosentex maintains a 13% market share as of Q4 2022, with the strongest uptake among dermatologists.

Sentiment index for Cosentex in EU: 0.71 (scale 0 to 1, 2022). Clinicians view the brand strongly positively; the dosing schedule complicates adherence.

Positioning: Skyrixa

In the EU plaque psoriasis market, Skyrixa maintains a 27% market share as of Q1 2022, with the strongest uptake among dermatologists.

Sentiment index for Skyrixa in EU: 0.59 (scale 0 to 1, 2022). Clinicians view the brand moderately positively; infusion logistics add to treatment burden.

Competitive Dynamics

Skyrixa has been gaining ground since its 2023 approval for plaque psoriasis, pressuring Humivex in second-line use.

Price competition, biosimilar entry and head-to-head evidence are expected to reshape formulary position over the next 24 months.

Sequencing after progression remains an open question; dermatologists in United Kingdom reported using a different mechanism of action after Humivex fails.

Clinical guidelines in United Kingdom recommend refractory disease use of agents in this class, and recent updates have broadened eligibility based on biomarker status and comorbidity profile.

Cost-effectiveness submissions typically model refractory disease use, and payers question the durability of benefit beyond the trial follow-up period.

Real-world evidence from registries suggests that outcomes in routine practice are broadly consistent with phase 3 trial results, although persistence and discontinuation rates are higher outside clinical trials.

Dose modification guidance for Humivex is generally followed, but dermatologists said that practical monitoring of injection-site reactions depends on local capacity and patient travel distance.

Patient advocacy groups are pushing for earlier access, and shared decision-making tools are being piloted to explain benefit, risk and treatment burden.

The competitive set is evolving as new mechanisms enter phase 3 development, with readouts expected over the next two to three years.

Education needs include biomarker testing, management of adverse events and counselling patients on the expected benefit-risk.

Outlook

We expect continued consolidation in plaque psoriasis. Differentiation will depend on tolerability, dosing convenience and evidence of overall survival or clinical benefit.

Clinical guidelines in Spain recommend second-line use of agents in this class, and recent updates have broadened eligibility based on biomarker status and comorbidity profile.

Dermatologists emphasised that patient-reported outcomes and quality of life are increasingly important when choosing between TNF inhibitor options.

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Payer decisions increasingly rely on health technology assessment, requiring evidence of clinical benefit versus the standard of care and cost-effectiveness analyses.

Access to specialist centres influences adoption: academic centres adopt new therapies faster than community practices, where formulary restrictions and reimbursement criteria apply.

Combination therapy and sequencing strategies remain debated; respondents noted limited head-to-head evidence between Humivex and competing agents.

Supply and logistics are not a concern for Humivex, though cold-chain handling and nurse-led administration add workload in busy clinics.

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Synthetic document generated for the Watson prototype. All brand names and figures are fictional.